

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction

for the supply of Fluconazole or Clotrimazole for the treatment of Vaginal Thrush (Vulvovaginal Candidiasis)

Patient Group Direction, version 006, issued 14 September 2026. Supersedes Version 005, 11 September 2026.

Owner and authorising organisation: Get Real Health Limited, company number 12744898, registered office Zenvite Health Unit 23, St. Asaph Business Park, St. Asaph, Denbighshire, LL17 0LJ. This Patient Group Direction is authorised on behalf of Get Real Health Limited by its Medical Director and its Head Pharmacist, whose signatures for this version are on the authorisation page at the end of the document. It applies to the registered healthcare professionals named under it by a pharmacy or clinic that has adopted it through a Get Real Health service agreement, at the premises stated in that adoption, and to no other organisation.

Summary of NICE / NICE CKS Guidelines for Vaginal Thrush (Vulvovaginal Candidiasis)

Overview

Vulvovaginal candidiasis is a common fungal infection caused by *Candida albicans*, affecting the vulva and vaginal tissues.

Assessment

- **Symptoms:** Vulval itching, vaginal discharge (typically thick, white, curdy), vaginal soreness, vulval erythema, and pain during intercourse or micturition.
- **Dysuria:** external stinging as urine passes over inflamed vulval skin is a symptom of thrush. Internal dysuria (pain inside the urethra or bladder), or dysuria of any kind with urinary frequency, urgency or fever, points to a urinary tract infection and is an exclusion under this PGD.

Management

- **First-line treatment:** Oral antifungal (fluconazole 150mg single dose) or topical antifungal (clotrimazole 500mg pessary single dose).



Red flags requiring referral or specialist advice

- **Recurrent infection:** 4 or more episodes per year - refer to GP for further investigation.
- **Immunocompromised patients:** Require careful management and GP referral.
- **Pregnancy:** Topical antifungal only; oral fluconazole contraindicated in pregnancy.

Patient Group Direction

for the supply of

Fluconazole 150mg capsule

for the treatment of

Vaginal Thrush (Vulvovaginal Candidiasis)

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.
PGD Indication	Treatment of uncomplicated vulvovaginal candidiasis in non-pregnant women aged 16 to 60 years

Inclusion criteria	Women aged 16 to 60 years Clinical diagnosis of uncomplicated vulvovaginal candidiasis Not pregnant or breastfeeding No contraindications
Exclusion criteria	Pregnancy Breastfeeding (relative contraindication due to insufficient data) Known hypersensitivity to fluconazole or azoles Concurrent use of terfenadine, astemizole, cisapride, pimozide, quinidine or erythromycin (risk of QT prolongation and torsades de pointes) First episode of symptoms (needs a diagnosis; refer) 4 or more episodes in 12 months, or 2 in the last 6 months (recurrent candidiasis; refer) Immunosuppression, or diabetes that is poorly controlled Abnormal or blood-stained vaginal bleeding, vulval ulcers, sores or blisters Lower abdominal pain, fever or systemic upset, or foul-smelling discharge Internal dysuria (pain felt inside the urethra or bladder on passing urine), or dysuria of any kind together with urinary frequency, urgency or fever: possible urinary tract infection, refer. External dysuria alone (stinging as urine passes over the sore vulva), with the other features of thrush, is not an exclusion. Possible exposure to a sexually transmitted infection, or a partner with an STI Aged under 16 or over 60 Severe hepatic impairment (Child-Pugh score > 9) Severe renal impairment (eGFR < 20 mL/min) History of QT prolongation or cardiac arrhythmias
Cautions	Hepatic impairment (mild to moderate) - monitor closely Renal impairment (mild to moderate) - dose adjustment may be needed Drug interactions: warfarin and other oral anticoagulants (increased anticoagulant effect), statins (increased statin levels), phenytoin (increased phenytoin levels), rifampicin (reduced fluconazole levels) Consider other medication interactions via BNF
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed.

	Document any advice given and the decision reached. Inform or refer to the GP as appropriate.
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Details of the medicine

Name, form and strength of medicine	Fluconazole 150mg capsule
Legal category	P (Pharmacy medicine) for women aged 16 to 60, which is the age range this PGD covers; POM outside it. A PGD is not legally required for a P sale; this arm is included so that the supply is assessed and recorded to the same standard as a POM supply.
Storage	Store below 25°C in a dry place. Keep out of reach of children.
Route/method of administration	Oral, swallowed whole
Dose and frequency	Single dose of 150mg orally
Quantity to be administered and/or supplied	1 capsule (150mg)
Maximum or minimum treatment period	Single dose; no further treatment required unless symptoms persist
Adverse effects	Common: nausea, abdominal pain, diarrhoea, headache Uncommon: rash, pruritus, dizziness Rare: hepatotoxicity (raised liver enzymes, hepatitis), anaphylaxis, Stevens-Johnson syndrome, toxic epidermal necrolysis

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: - that valid informed consent was given - name of individual, address, date of birth and GP - name of healthcare practitioner - name and brand of medication - date of supply dose, form and route quantity supplied - advice given, including advice given if excluded or declines treatment - details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made.

	A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)
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Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	Symptoms should resolve within 5-7 days. If symptoms persist or worsen, contact your GP. Wear cotton underwear and avoid irritants such as douches, scented products, and tight clothing. Avoid sexual intercourse for at least 5 days after treatment (pessary may damage condoms/diaphragms). If using the pessary, insert with fingers rather than the applicator to minimize damage to barriers. If you experience recurrent infections (4 or more per year), contact your GP for investigation. This is a single-dose treatment. No further doses are needed unless you are advised otherwise. Report any adverse effects to your healthcare provider or via the Yellow Card scheme.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mpg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance: Vulvovaginal candidiasis
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date	Expiry date		
14/9/26	31/7/27		

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or medicine.

This section must be signed by the superintendent or clinical lead responsible for this service. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff. To be completed by the adopting pharmacy. These are the adopting organisation's own signatories and must not be Get Real Health staff.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date



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Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025

Patient Group Direction
for the supply of
Clotrimazole 500mg vaginal pessary
for the treatment of
Vaginal Thrush (Vulvovaginal Candidiasis)

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must have completed training in working under Patient Group Directions, for example the CPPE PGD e-learning and e-assessment or an equivalent, and have been assessed as competent to work under a PGD before first working under this one; previous experience of working under a PGD is not required • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Treatment of uncomplicated vulvovaginal candidiasis in non-pregnant women aged 16 to 60 years
Inclusion criteria	Women aged 16 to 60 years Clinical diagnosis of uncomplicated vulvovaginal candidiasis No contraindications

	Able to insert pessary intravaginally
Exclusion criteria	Known hypersensitivity to clotrimazole or imidazoles First episode of symptoms (needs a diagnosis; refer) 4 or more episodes in 12 months, or 2 in the last 6 months (recurrent candidiasis; refer) Immunosuppression, or diabetes that is poorly controlled Abnormal or blood-stained vaginal bleeding, vulval ulcers, sores or blisters Lower abdominal pain, fever or systemic upset, or foul-smelling discharge Internal dysuria (pain felt inside the urethra or bladder on passing urine), or dysuria of any kind together with urinary frequency, urgency or fever: possible urinary tract infection, refer. External dysuria alone (stinging as urine passes over the sore vulva), with the other features of thrush, is not an exclusion. Possible exposure to a sexually transmitted infection, or a partner with an STI Aged under 16 or over 60 Pregnancy, known or suspected. The indication for this arm is non-pregnant women; refer to the GP or midwife.
Cautions	Pregnancy is an exclusion (above). Refer to the GP or midwife. Where a topical antifungal is prescribed in pregnancy the applicator should not be used. May damage latex condoms and diaphragms - advise alternative contraception for 5 days after use Ensure patient can retain pessary (abnormal anatomy, severe prolapse)
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate.

Details of the medicine

Name, form and strength of medicine	Clotrimazole 500mg vaginal pessary
Legal category	P (Pharmacy medicine) for women aged 16 to 60, which is the age range this PGD covers. A PGD is not legally required for a P sale; this arm is included so that the supply is assessed and recorded to the same standard as a POM supply. The conditions of the P licence (first

	episode, 2 or more episodes in 6 months, pregnancy, under 16 or over 60, and the other seek-advice features) are carried in the exclusion criteria above.
Storage	Store below 25°C in a dry place. Keep out of reach of children.
Route/method of administration	Intravaginal insertion at bedtime
Dose and frequency	Single 500mg pessary inserted intravaginally at night
Quantity to be administered and/or supplied	1 pessary (500mg)
Maximum or minimum treatment period	Single application; treatment completed in one night
Adverse effects	Common: local vaginal irritation, burning sensation, vaginal erythema Uncommon: vaginal candidiasis (rebound infection)

Yellow card reporting	Report suspected adverse effects via https://yellowcard.mhra.gov.uk and inform the GP as appropriate.
Records to be kept	Record the following information: • that valid informed consent was given • name of individual, address, date of birth and GP • name of healthcare practitioner • name and brand of medication • date of supply dose, form and route quantity supplied • advice given, including advice given if excluded or declines treatment • details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: • Adults aged 18 years and over - keep records for 8 years • Children aged under 18 years - keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)

Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	Symptoms should resolve within 5-7 days. If symptoms persist or worsen, contact your GP. Wear cotton underwear and avoid irritants such as douches, scented products, and tight clothing. Avoid sexual intercourse for at least 5 days after treatment (pessary may damage condoms/diaphragms). If using the pessary, insert with fingers rather than the applicator to minimize damage to barriers. If you experience recurrent infections (4 or more per year), contact your GP for investigation. This is a single-dose treatment. No further doses are needed unless you are advised otherwise. Report any adverse effects to your healthcare provider or via the Yellow Card scheme.

Key references

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Valid from date	Expiry date		
14/9/26	31/7/27		

PGD adoption and authorisation by the employer/clinical lead

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Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Change history to version 001

Later versions are recorded in the Version and change record at the end of this document.

Version number	Change details	Date
001	Development & issue of new PGD	1st November 2025





Version and change record

Version: v006

Issued: 14 September 2026

Supersedes: Version 005, 11 September 2026

Valid from: 14 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Identity and training, applied to every Get Real Health PGD on 14 September 2026 after two adopting pharmacies wrote: the Get Real Health logo is in the page header; the organisation that owns and authorises this direction, Get Real Health Limited, is named under the strapline with its company number and registered office; the requirement that practitioners must previously have used a PGD is replaced by a requirement for PGD training and a competence assessment, so that a newly registered pharmacist or technician who has completed the training may work under it; and the earlier change history table is headed with the versions it covers.

Previous version records

Version: v005

Issued: 11 September 2026

Supersedes: Version 004, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Decision 40: the dysuria exclusion in both arms narrowed to internal dysuria, or dysuria with urinary frequency, urgency or fever; external dysuria alone with the other features of thrush no longer excludes, and the guidance summary states the distinction.

Decisions of the authorising signatories, 11 September 2026: the questions raised by the reviews of 10 and 11 September were put to the Medical Director and answered; each answer is written into this document above and, where the answer set a rule, into the consultation tool. Text drafted from a source for the Head Pharmacist to confirm is marked as such in the change lines.

Previous version records

Version: v004

Issued: 11 September 2026

Supersedes: Version 003, 11 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Indication and inclusion rows in both arms now say 16 to 60, matching the exclusion row and the P licence; they said 16 to 65.

Cover and arm headings corrected from administration to supply.

Follow-up advice no longer tells the patient to complete a course; both products are single dose.



Pessary arm: pregnancy is an exclusion with a referral, as its indication (non-pregnant women) says; the caution that allowed the pessary in pregnancy with finger insertion is replaced.

Pessary legal category states which P licence conditions are carried in the exclusion criteria.

Broken NICE MPG2 link corrected.

Wording corrections from the tool alignment and adversarial review of the consultation tools, 11 September 2026. Each correction resolves a contradiction inside the document or a plain error, taking the safer reading; no indication is widened, no dose raised and no exclusion removed. Questions that need a clinical decision are recorded separately and are not changed here.

Previous version records

Version: v003

Issued: 11 September 2026

Supersedes: Version 002, 10 September 2026

Valid from: 11 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Legal categories corrected: fluconazole 150 mg and clotrimazole 500 mg pessary are P medicines for women aged 16 to 60, the range this PGD covers. Version 002 called both POM.

Structural clean-up applied to every live Get Real Health PGD on 11 September 2026, following the adversarial review of 10 September: the adopting pharmacy's authorisation block is blank, for the pharmacy's own signatories; one signed authorisation page for this version replaces the earlier signature blocks; every validity cell states this version's dates (valid from 11 September 2026, expiry 31 July 2027); narration about earlier versions, the consultation tool and individual customers is removed from the body of the document; the change records of earlier reissues are carried below as previous version records. No clinical criterion changes unless listed.

Previous version records

Version: v002

Issued: 10 September 2026

Supersedes: Version 001, 1 November 2025

Valid from: 10 September 2026. Expiry: 31 July 2027. These dates govern this version and supersede any earlier date printed elsewhere in this document.

Changes in this version:

Validity: this version is valid from 10 September 2026 and expires on 31 July 2027. The signed master previously stated no period of validity, or an expiry of 31 October 2026; those are superseded.

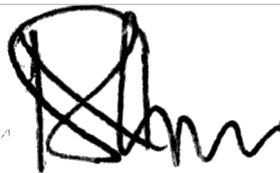
Fluconazole arm: quinidine and erythromycin added to the QT exclusion, as the SmPC contraindicates both.

Both arms: the red flags in the guidance summary and the Canesten SmPC's own 'seek medical advice' list (first episode, recurrent infection, immunosuppression, abnormal bleeding, ulcers, abdominal pain or dysuria, fever, possible STI, under 16 or over 60) were not exclusions in either arm. Added to both.



Signed on behalf of Get Real Health

Version v006, 14 September 2026.

Name	Qualification	Signature	Date
Nitin Shori	Medical Director GMC: 6047293		14 September 2026
Chris Pilkington	Head Pharmacist GPhC: 2046322		14 September 2026

Both authorising signatories reviewed this version together on 14 September 2026 and gave their authorisation for it to be issued. Signatures were applied digitally on their joint instruction, which is the established process for Get Real Health PGDs. This version is not valid without both signatures.